

Missing Patient Flow Chart

Adult patient missing from ward/department
Local search has been undertaken (appendix 1)

Has the patient 'absconded'? i.e. are they detained or likely to be detained under the Mental Health Act, the Deprivation of Liberty Safeguards, or are they subject to a public health order or subject of custody (Police or Prison)?

N

Is the patient at risk of immediate or serious **medical** complications that may be life threatening or life changing?

Y

This does not include minor injury/illness. Be prepared to provide a clear rationale to the Police

N

Does the patient lack mental capacity to consent to / decline care and treatment? This can be documented or **based on a reasonable belief**

Y

Provide a clear rationale to the Police i.e. Patient delirious / confused / has dementia and is unlikely to have capacity
May inability to safely interact with other or in an unknown environment

N

Do you believe that the patient is involved in serious crime as either a perpetrator or victim?

Y

Examples include; Kidnap, False Imprisonment, Forced Marriage Significant Domestic Violence

N

Do you believe there is an **immediate** threat to life and limb as a result of the patient being missing? To the patient or others

Y

Examples include; deliberate harm, vulnerability due to disability or infirmity, dependent on the assistance of a carer/relative.

N

Do you believe the patient to be at risk of exploitation or abuse including patients with significant learning difficulties or where there are on-going safeguarding investigations?

Y

Outcome: Medium/High Risk

Outcome: Low Risk

1. Ring patients telephone number on their notes
2. Inform Nurse in Charge, Matron and Duty manager and security
3. Unable to locate or make contact with the patient, or the patient refuses to return to hospital? Complete discharge letter and notify the GP, Next of Kind and other relevant professional agencies, district nurse, social worker etc.
4. Document all your actions and assessments in the patient's notes.
5. If you receive new information that may change the outcome of their risk assessment, **escalate**.

1. Ring patients telephone number on their not
2. Notify 2222 of an Missing Adult Patient from your area
3. Contact patient's next of kin/significant other to see if they have had contact with the patient.
4. Inform Nurse in Charge & Matron
5. If unable to make contact or the patient is refusing to return, notify the Police advising them of the risk level and your concerns. The Police will ask you the questions overleaf, be prepared to have the information available. The police will want to speak to the last people (doctor and nurse) that saw the patient. They may need to remain in the area or leave a telephone number to speak to the Police.
6. Liaise with clinical staff throughout regarding clinical concerns of the patient.
7. Document your actions in patient's notes and complete DATIX incident form ticking the safeguarding box.

Consult the UHL Missing Patient Policy section 6.5.and 6.6 for further information

Police Questions for member of staff reporting a patient missing – continue on separate page if necessary

Police Question	UHL Response
1. Patient details including Full Name, Date of birth, Address, Telephone numbers	[INSERT PATIENT STICKER]
2. Detail the brief circumstances of going missing / absent?	
3. Where, when last seen and by whom?	
4. What has been done so far to trace this individual, and if not why not?	
5. What is the description of the person including clothing?	
6. Is this significantly out of character, if so why?	
7. What is the specific concern in this instance (if any)?	
8. Any last known intentions or preparations made prior to going absent/missing?	
9. Have they taken any personal items with them	
10. Do they have a mobile phone in their possession? (Network and Phone Number required)	
11. Identify any place they may go to and why: specific interest or to find solace/peace and quiet?	
12. Are they subject to any Mental Health Section- if so what and why?	
13. Are there any specific medical needs that require medication and if this isn't available what are the effects and timescales?	
14. Are they likely to be a victim of crime – if so why?	
15. Are they likely to be the victim of abuse – if so explain? (Domestic / Sexual / Racial / Bullying / Homophobic)	
16. Are they currently at risk of sexual exploitation or on Child Protection register – If so from whom?	
17. Are they likely to self-harm, attempt suicide– give details including last known attempt	
18. Have they been exposed to harm in any previous missing episode – if so provide details	
19. Do they pose a danger to themselves or any other persons?	
20. Does the missing person have a current or previous history of drug or alcohol abuse (give details)?	
21. What access do they have to money?	
22. Is there any other information relevant to their absence that may affect or influence a supervisor's risk assessment?	
23. Contact Number	Provide ward/department details as well as the site Duty Manager
Information to obtain from the Police by UHL Staff	
24. Crime Reference Number:	
25. Police Severity Rating	